

STANDARD OPERATING PROCEDURE

TRANSPORTATION OF HIGH-RISK PREGNANCIES TO MCHs / FUNCTIONAL FRUs



Topic: 3

Tagging Calls to the Right FRU



108 Emergency Call Centre

The 108 Emergency Call Centre shall ensure that referral calls from the concerned blocks are linked to the respective tagged FRUs, as indicated in **Annexure I**.



Referring Facilities (Block Level)

Referring facilities at the block level (**SC/PHC/CHC/SDH**) shall initiate referrals to the tagged FRU, unless diversion is required due to unavoidable circumstances.

Confirming High-Risk Status and Dispatch



Confirmation of High-Risk Status

If the call from HRP is given to 108 from home by ASHA/or attendant of pregnant women, then confirmation of high risk status will be made by ensuring the RED card availability with delivery case or by enquiring of any complicated condition/ danger sign condition. In such cases, the delivery case is to be referred to the functional FRU as per the tagged block list.



Call Centre Action

The call centre shall dispatch the ambulance and simultaneously send an SMS to the referral nodal officer of the tagged facility to ensure advance preparedness for receiving the patient.

Who Signs the Referral Slip



Receiving the Patient

The 108 EMAS /Janani driver shall receive the patient along with a properly filled referral slip signed by the Medical Officer of the referring facility.



Below CHC Level (PHC / Sub Centre)

In facilities below CHC level, ie., PHC, the Medical Officer/AYUSH doctor who ever has treated the case, shall sign the referral slip. In case of non-availability of Medical Officer, senior nursing officer/ on duty nursing officer of the facility/and HW(F) in case of sub center shall sign the referral slip.



Night Hours

If the referral is done during night hours the available MO /Nursing officer will give the referral slip to the maternity case while referring.

HANDOVER

Receiving the Patient at the Facility



Receiving Facility Preparedness

The receiving MCHs/FRUs shall ensure preparedness for receiving referred high-risk pregnancy (HRP) cases and maintain effective communication with the referring facility and the 108 EMAS/Janani driver.



Recording the Handover

The 108 Janani driver/EMAS shall hand over the patient at the referred facility along with the referral slip and handover the referral case to the facility and record the details in the handing over app by the 108 EMAS/Janani driver.



Photo & Digital Signature

While handing over, both photograph and digital signature will be obtained through the app from the person to whom patient will be handed over at the facility level.



District Monitoring

District authorities shall monitor adherence to the tagging mechanism and review referral patterns during routine review meetings.

Sensitization, Retagging & Reporting



Sensitization of Field Staff

All 108 Janani drivers/EMAS/ Service providers / NHM PMU staff shall be sensitized on the SOP for referral of high-risk pregnancies at district and block level



Retagging of Facilities

In case of any change in the functionality status of FRU, transfer of specialist manpower, any infrastructural related issues OT or Labour Room, the district will inform to State for retagging of the facilities.

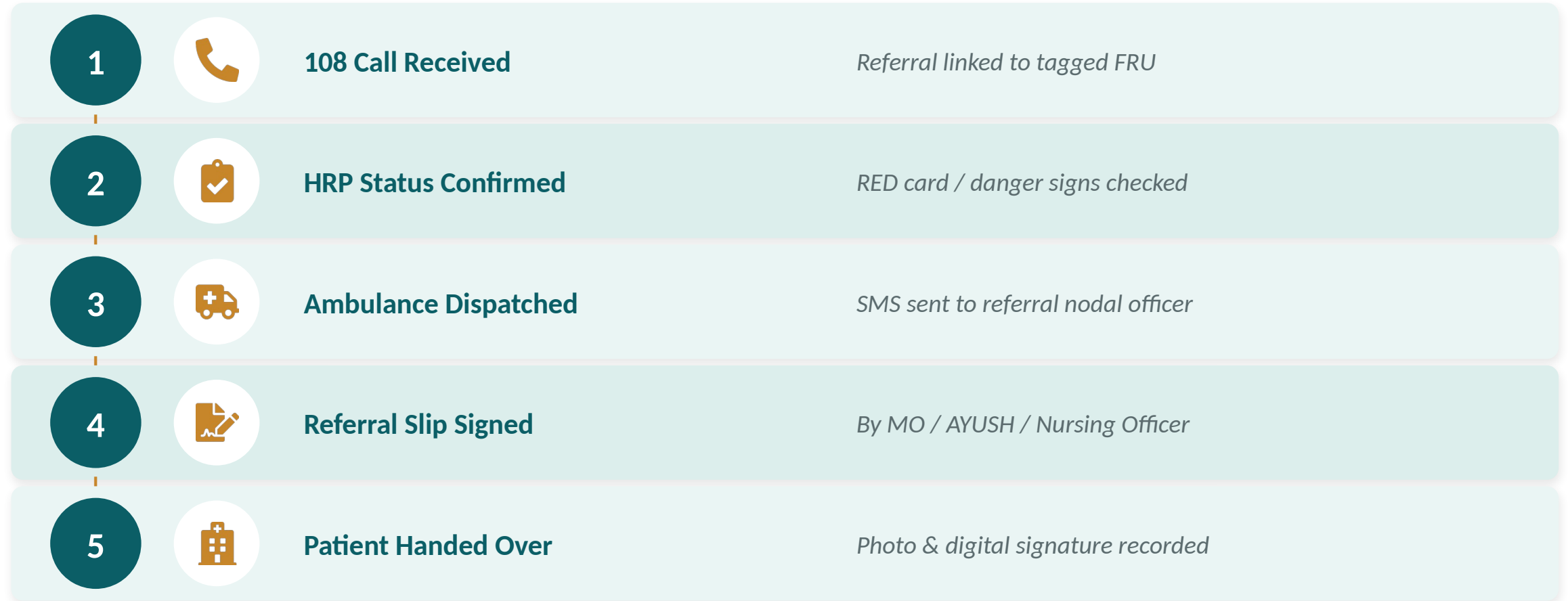


Monthly Status Update

The update on the FRU status is to be given to State on the monthly basis by CDM& PHO.

PROCESS AT A GLANCE

Referral Flow: Call to Handover





Topic:4

HRP condition in MCP card

ପ୍ରତ୍ୟେକ ଗର୍ଭବତୀ ମହିଳାଙ୍କ ପାଇଁ
(ମହିଳା ସ୍ୱାସ୍ଥ୍ୟକର୍ମୀ ଦ୍ୱାରା)

ଗର୍ଭବତୀ ମହିଳାଙ୍କର ବ୍ୟକ୍ତିଗତ ଆବଳନ

କ୍ର	କ୍ଲିନିକାଲିକ ସ୍ୱଚ୍ଛତା	ଉତ୍ତର ଅପରାଧ / ସାମାଜିକ ଅପରାଧ
କ.୧	ଗ୍ରାମର ପ୍ରକାର	ଉତ୍ତର ଅପରାଧ / ସାମାଜିକ ଅପରାଧ
ଖ.	ଶାରୀରିକ ଅବସ୍ଥା (ବିପଦ ସଙ୍କଟ ଅବସ୍ଥା)	ଉଚିତ ନାମରେ "✓" ଚିହ୍ନ ଦିଅନ୍ତୁ
ଘ.୧	ଅତୀତ ଗର୍ଭର ସ୍ୱଚ୍ଛତା (ପ୍ରଥମ ପରିବର୍ତ୍ତନ ବେଳେ ଆବଳନ କରନ୍ତୁ)	ପ୍ରଥମ ପରିବର୍ତ୍ତନର ତାରିଖ
ଘ.୧.୧	୪ ରୁ ଅଧିକ ଥର ଗର୍ଭବତୀ ହୋଇଥିଲେ କି ?	ହଁ / ନା
ଘ.୧.୨	ଆଗରୁ ଅସ୍ତ୍ରୋପଚାର କରାଯାଇଥିବା ପ୍ରସବ / ଅନ୍ୟ କୌଣସି ଗର୍ଭାଶୟ କରାଯାଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୩	ପୂର୍ବରୁ L.B.W. ଶିଶୁ / ପ୍ରିମର୍ ଶିଶୁ / I.U.G.R. ଶିଶୁ ଜନ୍ମ ହୋଇଥିଲେ କି ?	ହଁ / ନା
ଘ.୧.୪	ଆଗରୁ କେବେ ନବଜାତ ଶିଶୁର ମୃତ୍ୟୁ / ଗର୍ଭରେ ମୃତ୍ୟୁ / ଜନ୍ମ ସମୟରେ ଶ୍ୱାସରୁଦ୍ଧ କରାଯାଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୫	ଅତୀତରେ ପ୍ରସବ ପୂର୍ବରୁ ରକ୍ତସ୍ରାବ / ପ୍ରସବ ପରବର୍ତ୍ତୀ ରକ୍ତସ୍ରାବ କରାଯାଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୬	ମଧୁମେହ / ହୃଦ୍ରୋଗ / କବ୍ଜ କରାଯାଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୭	ଅତୀତରେ ଗର୍ଭାବସ୍ଥାରେ ବାତ ମାରିଥିବା / ଉଚ୍ଚ ରକ୍ତଚାପ ଥିବା କି ?	ହଁ / ନା
ଘ.୧.୮	ଅତୀତରେ ଅତିକମ୍ପରେ ୩ ଥର ସ୍ୱତଃ ଗର୍ଭପାତ ହୋଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୯	ଅତୀତରେ ଯକ୍ଷ୍ମା କିମ୍ବା ଯକ୍ଷ୍ମା ହୋଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୧୦	ଦୀର୍ଘ ଦିନର ବନ୍ଧାବନ୍ଧୁ ପରେ ଗର୍ଭବତୀ କିମ୍ବା କୃତ୍ରିମ ଉପାୟରେ ଗର୍ଭବତୀ ହୋଇଥିଲେ କି ?	ହଁ / ନା
ଘ.୧.୧୧	ପୂର୍ବ ପ୍ରସବ କରାଯାଇଥିବା କି ?	ହଁ / ନା
ଘ.୧.୧୨	HIV+ ଚିହ୍ନିତ ହୋଇଥିଲେ କି ?	ହଁ / ନା
ଘ.୨	ବର୍ତ୍ତମାନର ଗର୍ଭାବସ୍ଥା	
ଘ.୨.୧	ଏହି ଗର୍ଭବତୀର ବୟସ ୧୮ ବର୍ଷରୁ କମ୍ ନା ୩୫ ବର୍ଷରୁ ଉର୍ଦ୍ଧ୍ୱ କି ?	ହଁ / ନା
ଘ.୨.୨	ଆକାସିନିଆ / ଉଚ୍ଚ ଶିଶୁନି ରୋଗ ଅଛି କି ?	ହଁ / ନା
ଘ.୨.୩	ଆଇଭଏଚ୍ ରୋଗ ଅଛି କି ?	ହଁ / ନା
ଘ.୨.୪	ଗର୍ଭବତୀଙ୍କର Rh -ve କି ?	ହଁ / ନା
ଘ.୨.୫	ଉଚ୍ଚତା ୧୪୦ ସେ.ମି. ରୁ କମ୍ ଅଛି କି ?	ହଁ / ନା
ଘ.୨.୬	ଓଜନ ୪୦ କେ.ଜି.ରୁ କମ୍ ?	ହଁ / ନା
ପ୍ରତ୍ୟେକ ଗର୍ଭବତୀଙ୍କର ଏକାଧାରରେ ବିପଦ ସଂକ୍ରମଣ ଅବସ୍ଥାର ଆବଳନ ୧		୧ମ ଏକାଧାର (୧୨ ସପ୍ତାହ ମଧ୍ୟରେ) ୨ୟ ଏକାଧାର (୧୫-୨୬ ସପ୍ତାହ ମଧ୍ୟରେ) ୩ୟ ଏକାଧାର (୨୮-୩୫ ସପ୍ତାହ ମଧ୍ୟରେ) ୪ର୍ଥ ଏକାଧାର (୩୬ ସପ୍ତାହ ପରେ ମଧ୍ୟରେ)
ଘ.୨.୭	ହିମୋଗ୍ଲୋବିନର ମାତ୍ରା ଲେଖନ୍ତୁ (g/dl)	
ଘ.୨.୮	ଉଚ୍ଚତାପର ମାପ ଲେଖନ୍ତୁ	
ଘ.୨.୯	ପରିସ୍ରାରେ ଶର୍ଦ୍ଦିଆ/ଆଇସ୍/ନିମ୍ନ / ASB+ve ଅଛି କି ? ହଁ/ନା	
ଘ.୨.୧୦	ଗର୍ଭାବସ୍ଥାରେ ମାଲେରିଆ / ଜ୍ୱର ଅଛି କି ? ହଁ/ନା	
ଘ.୨.୧୧	ପ୍ରଥମ ଗ୍ରାହଣେଷର ଠାରୁ ମାସିକ ଓଜନ ୫୦୦ ଗ୍ରାମରୁ କମ୍ ନା ବେଳକୁ ଅଧିକ ବଢ଼ିଛି କି ? ହଁ/ନା	
ଘ.୨.୧୨	ହେପାଟାଇଟିସ୍ ବି / କଣ୍ଟିକ୍ସ ଅଛି କି ? ହଁ/ନା	
ଘ.୨.୧୩	ଗର୍ଭାବସ୍ଥାରେ HIV+ / RTI / STI ଅଛି କି ? ହଁ/ନା	
ଘ.୨.୧୪	ଯୋନିରୁ ରକ୍ତସ୍ରାବ ହେଉଛି କି ? ହଁ/ନା	
ଘ.୨.୧୫	Adult MUAC ବାହୁ ମାପ ଲେଖନ୍ତୁ*	

* ବାହୁ ମାପ ୨୩ ସେ.ମି.ରୁ କମ୍ ହେଲେ ପୁଷିକାଳ କୁହାଯିବ

ଗର୍ଭବତୀ ମହିଳାଙ୍କର ବ୍ୟକ୍ତିଗତ ଆବଳନ

ବିପଦ ସଙ୍କଟ ଅବସ୍ଥା ପରିବର୍ତ୍ତନର ତାରିଖ	୧ମ ଏକାଧାର (୧୨ ସପ୍ତାହ ମଧ୍ୟରେ) ତା.	୨ୟ ଏକାଧାର (୧୫-୨୬ ସପ୍ତାହ ମଧ୍ୟରେ) ତା.	୩ୟ ଏକାଧାର (୨୮-୩୫ ସପ୍ତାହ ମଧ୍ୟରେ) ତା.	୪ର୍ଥ ଏକାଧାର (୩୬ ସପ୍ତାହ ପରେ ମଧ୍ୟରେ) ତା.
ଘ.୨.୧୬ ପି.ଆର୍.୫.ଏମ୍/ଲିଭିଙ୍ଗ୍ ଅଛି କି ? ହଁ/ନା				
ଘ.୨.୧୭ ଦୂର୍ଗନ୍ଧଯୁକ୍ତ ଯୋନିସ୍ରାବ ହେଉଛି କି ? ହଁ/ନା				
ଘ.୨.୧୮ ଗର୍ଭସ୍ଥ ଶିଶୁର ଚଳପ୍ରଚଳ କମି ଯାଇଛି କି ? ହଁ/ନା (୧୨ ସପ୍ତାହ ଭିତରେ ପିଲା ୧୫ ଥର ଚଳପ୍ରଚଳ କରୁଛି କି ନାହିଁ)				
ଘ.୨.୧୯ ଅନିୟମିତ ଯନ୍ତ୍ରଣାଯୁକ୍ତ କଣ୍ଠାକୃତ୍ ହେଉଛି କି ? ହଁ/ନା				
ଘ.୨.୨୦ ଗର୍ଭାଶୟର ଆକାର ଅସ୍ୱାଭାବିକ (ଗର୍ଭାଶୟର ଆକାର ଗର୍ଭସ୍ଥ ଶିଶୁର ବୟସ ଠାରୁ ୪cm ଅଧିକ ବା କମ୍) ଜାଣି ବୁଝି ହୋଇଛି କି ? ହଁ/ନା				
ଘ.୨.୨୧ ପ୍ରଥମ ଚିନିମାରେ ଚଳିପେରେ ଅତ୍ୟଧିକ ଯନ୍ତ୍ରଣା ହେଉଛି କି ? ହଁ/ନା				
ଘ.୨.୨୨ ସମସ୍ତା ପ୍ରସବ ତାରିଖ ୭ ଦିନ ରୁ ଅଧିକ ଗତିଛି କି ? ହଁ/ନା				

S.L. No	High risk Condition (History)	S.L. No	High risk Condition (Current pregnancy)
1	Previous LSCS	19	Geographically hard to reach area
2	Any other Surgery involving Uterus	20	Grand Multi (Morethan 4 Preganancy)
3	Previous H/O LBW	21	Age less than 18 years
4	Previous H/Opreterm	22	Age morethan 35 years
5	Previous H/O IUGR	23	Thallasemia
6	H/O Newborn Death	24	Sickle cell Disease
7	H/O Still Birth	25	Hypothyroidism
8	H/O Birth asphyxia related death	26	Rh(-ve)
9	H/O APH in previous Pregnancy	27	Height less than 140 cms
10	H/O PPH in previous Pregnancy	28	Weight less than 40 kgs
11	H/O Diabetes	29	Severe Aneamia
12	H/O Hypertension	30	Hypertensive disorder of pregnancy
13	H/O Kidney disease	31	Gestational Diabetes Mellitus
14	H/O Hypertensive disorder of pregnancy in previous pregnancy	32	Asymptomatic Bacteriuria
15	H/O Minimum 3 spontaneous Abortion	33	Malaria
16	H/O TB or Leprosy	34	Weight gain less than 500gms or more than 3 kgs in a month after 1st trimester
17	H/O conceive after long period/ IVF	35	Hepatitis B
18	H/O High risk during previous pregnancy	36	HIV (+ve)
		37	STI/RTI
		38	Vaginal Bleeding
		39	Adult MUAc less than 23 cms
		40	PROM/ Leaking
		41	Foul smelling Vaginal Discharge
		42	Decrease Fetal Movement
		43	Irregular preterm contraction
		44	Abnormal Fundal height (morethan 3 from estimated Gesttaional age)
		45	extreme pain in lower abdomen during 1st trimester
		46	Post term (more than 7 days from EDD)

HRP Condition to be evaluate in 1st visit

S.L. No	High risk Condition (History)	S.L. No	High risk Condition (Current pregnancy)
1	Previous LSCS	19	Geographically hard to reach area
2	Any other Surgery involving Uterus	20	Grand Multi (Morethan 4 Preganancy)
3	Previous H/O LBW	21	Age less than 18 years
4	Previous H/Opreterm	22	Age morethan 35 years
5	Previous H/O IUGR	23	Thalassemia
6	H/O Newborn Death	24	Sickle cell Disease
7	H/O Still Birth	25	Hypothyroidism
8	H/O Birth asphyxia related death	26	Rh(-ve)
9	H/O APH in previous Pregnancy	27	Height less than 140 cms
10	H/O PPH in previous Pregnancy	28	Weight less than 40 kgs
11	H/O Diabetes		
12	H/O Hypertension		
13	H/O Kidney disease		
14	H/O Hypertensive disorder of pregnancy in previous pregnancy		
15	H/O Minimum 3 spontaneous Abortion		
16	H/O TB or Leprosy		
17	H/O conceive after long period/ IVF		
18	H/O High risk during previous pregnancy		

HRP Condition to be evaluate in every visit/Subsequent visit

S.L. No	High risk pregnancy	1 st ANC	2 nd ANC	3 rd ANC	4 th ANC
1	Hemoglobin (Severe Aneamia), Write the value				
2	Blood pressure (For Hypertensive disorder of pregnancy)				
3	Is there Albumin, Sugar or ASB + in urine (Yes/ NO)				
4	Malaria during pregnancy/ Fever (Yes/ No)				
5	Weight gain less than 500gms or more than 3 kgs in a month after 1st trimester. (yes/ No)				
6	Hepatitis B / Jaundice (Yes/ No)				
7	HIV +/STI+/RTI+ During Pregnancy (Yes/ No)				
8	Vaginal Bleeding during pregnancy				
9	Adult MUAC (<23 cm)				
10	PROM/ Leaking				
11	Foul smelling Vaginal Discharge				
12	Decrease Foetal Movement (15 movements/12 Hours)				
13	Irregular/painful contraction				
14	Abnormal Fundal height (more than 4cm from estimated Gestational age)				
15	Extreme pain in lower abdomen during 1st trimester				
16	Post term (more than 7 days from EDD)				

PMSMA and MCP card HRP alignment (1/2)

S.L. No	PMSMA HRP Condition	MCP card HRP
1	HIV	HIV
2	Syphilis	Syphilis
3	Severe Anemia	Hemoglobin less than 7gm%
4	PIH	BP >140/90mmhg, Urine albumin
5	GDM	Urine sugar, OGTT
6	Hypothyroidism	Thyroid problem
7	Tuberculosis	H/O TB
8	Malaria	Malaria
9	Previous LSCS	Previous LSCS
10	Cephalo-Pelvic Disproportion	
11	BOH	Prev. LBW, Preterm, IUGR, newborn deaths, Previous H/O APH , H/O PPH, more than 3 abortion, H/O PIH or eclampsia, primary / Secondary infertility, IVF
12	Twins/Multiple Pregnancy	
13	Hepatitis B	Hepatitis B

PMSMA and MCP card HRP alignment (2/2)

S.L. No	PMSMA HRP Condition	MCP card HRP Alignment
14	Abnormal FHR	
15	Teenage Pregnancy	If pregnancy less than 18 years
16	High Fever	Fever
17	RTI/STI	RTI/STI
18	H/O Still Birth	Previous Still birth
19	Congenital Malformation	
20	Negative Blood Group	Rh -ve
21	Early primi	If pregnancy less than 18 years
22	Elderly Primi	If 1 st pregnancy more than 35 years
23	Grand multipara	More than 4 pregnancy
24	Short Stature	Height less than 140cms
25	Others Specify	Geographically Hard to reach area, Prev. Surgery involving Uterus, H/O Diabetes, Heart disease, Kidney disease, H/O Leprosy. Any Pregnancy more than 35 years, Thalassemia, SCD, weight less than 40kgs, ASB, inappropriate wt gain , MUAC Less than 23cm

The expert committee unanimously recommended the following investigation for all ANC cases at the facility level:

CBC	ABO Grouping and RH Typing	TSH	OGTT/GCT
HIV	HBsAg	HCV	VDRL
	Gazelle Test (Haemoglobinopathies)	Urine ME/RE	

Following investigations are recommended for every delivery cases at the time of admission for delivery at facility:

CBC

Random Blood
Sugar

Urine ME/RE

LFT/RFT

ECG (if
available at
facility)

Follow up Tracking for HRP

once every 4
weeks until 24
weeks gestation

once every 2
weeks from 24 to
36 weeks
gestation

weekly thereafter
until delivery.

Patients must
report
immediately if any
discomfort or
danger signs
manifest